Which treatment this guide covers This guide covers lorlatinib alone for ALK-positive advanced non-small-cell lung cancer. The available Irish NCCP v3 document lists use after particular earlier ALK medicines. The current European licence also includes people who have not had an ALK inhibitor. The team must confirm your treatment setting and the current Irish protocol and access arrangements before prescribing; the source versions currently disagree.

NCCP 00570 · VERSION 3

Lorlatinib therapy

Lorlatinib is taken once daily continuously, with no planned rest week. Treatment continues while it is helping and side effects remain acceptable; there is no fixed number of treatment cycles.

BASELINE → TREATMENT → FOLLOW-UP

Checks during lorlatinib treatment

Monthly monitoring is not a chemotherapy cycle. Baseline coagulation is separate from routine monthly tests. Scans and extra checks follow the Consultant plan.

Check	Before starting	First four weeks	Weeks 5–8	Ongoing treatment	Hold / change / completion
Treatment and benefit	Confirm ALK-positive cancer, treatment line and access	Daily tablets; no rest week	Continue daily if authorised	Review benefit, harms and preferences	Team decides hold, switch or stop
Lipids and glucose	Baseline lipids and glucose	Lipids at weeks 2 and 4	Lipids at week 8	Lipids thereafter as appropriate; glucose as indicated	Review lipid/glucose medicines and recovery
Blood and pancreatic tests	FBC, kidney/liver profile, clotting, amylase/lipase	FBC, kidney/liver and pancreatic enzymes monthly	Continue monthly checks	Monthly or additional tests as indicated	Follow abnormal results and symptoms
Heart and blood pressure	ECG and BP; LVEF if cardiac risk warrants	BP at two weeks then at least monthly; ECG monthly	Continue ECG and BP monitoring	Earlier cardiac assessment for symptoms; LVEF if indicated	Restart only after source recovery criteria and specialist review
Symptoms and interactions	Baseline cognition, mood, medicines and support needs	Report changes promptly; check adherence and interactions	Review CNS, breathing, nerve and metabolic effects	Symptoms can occur at any time; scans per Consultant plan	Follow unresolved effects and contraception intervals

THE WHOLE TREATMENT JOURNEY

Lorlatinib therapy: treatment and checks

ALK-positive advanced NSCLC · specified later-line NCCP indication; separate licensed first-line context

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Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First four weeks	Weeks 5–8	Ongoing treatment	Hold / change / completion
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What each check is for

1 Confirm source and setting

Before starting

Sources: NCCP 00570 · SMPC

2 Early metabolic checks

Weeks 2, 4 and 8

Sources: NCCP 00570 · SMPC

3 Monthly review

During continuous therapy

Sources: NCCP 00570 · SMPC

4 Hold or change

Whenever toxicity or progression requires

Sources: NCCP 00570 · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.24 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending